Batterjee Medical College
For Science and Technology

Psychiatry · Clinical Lecture

Mood Disorders

The depressive and bipolar spectrums — definitions, criteria, differential diagnosis, suicidality, work-up, and modern treatment.

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Learning Objectives (1 of 2)

Mapped to the BMC Knowledge / Skill / Value framework. Prerequisite: psychiatric history taking and the mental status examination.

Knowledge & understanding

- Distinguish sadness from a depressive disorder
- Name the depressive and bipolar disorders and their DSM-5-TR criteria
- Apply SIG-E-CAPS and the mania / hypomania thresholds

Skills

- Take a mood history and screen every depressed patient for past mania
- Apply DSM-5-TR criteria to a real presentation
- Perform a structured suicide risk assessment

Values & attitudes

- Treat depression as a systemic, potentially lethal illness
- Ask about suicide directly — it is a duty, not a risk
- Diagnose within the patient's cultural and religious frame

Learning Objectives (2 of 2)

Knowledge & understanding

- Outline the monoamine, HPA-axis and neuroplastic models
- Recognise the specifiers that change management
- Describe course, prognosis and the evidence for each treatment

Skills

- Order and interpret the work-up that excludes organic mimics
- Separate grief, adjustment disorder and major depression
- Match treatment intensity to illness severity

Values & attitudes

- Involve the family as an ally in adherence and relapse prevention
- Use non-stigmatising language in the record
- Give prognosis honestly, without fatalism

Why this matters to you: mood disorders are the commonest psychiatric presentation in every specialty, and the suicide risk they carry is assessed by whoever is in front of the patient.

Sadness vs. depression

Sadness

- Normal, universal human emotion
- Proportionate to a trigger; transient
- Self-esteem and function preserved
- Resolves without treatment

Depression (disorder)

- Pervasive low mood and/or anhedonia
- Persistent (≥ 2 weeks) and out of proportion
- Impairs social and occupational function
- Often needs treatment

A mood is a sustained, pervasive emotional climate that colours perception of the world — distinct from a brief, reactive affect. Mood disorders are primary disturbances of that climate.

Types of mood disorders

Depressive spectrum — one pole

- Major Depressive Disorder (MDD)
- Persistent Depressive Disorder (dysthymia)
- Premenstrual Dysphoric Disorder (PMDD)
- Disruptive Mood Dysregulation Disorder (DMDD)
- Substance/medication- and medical-condition-induced

Bipolar & related spectrum — both poles

- Bipolar I disorder
- Bipolar II disorder
- Cyclothymic disorder
- Substance/medication- and medical-condition-induced

Placing a patient in the right family changes treatment completely — antidepressant monotherapy can destabilise a bipolar patient.

Mental health status in Saudi Arabia

Epidemiological picture

- Depression and anxiety are among the most common mental disorders nationally
- Saudi National Mental Health Survey: substantial lifetime prevalence, most untreated
- A young population — early-onset disorders carry a large disability burden

Care & barriers

- Stigma and somatic presentation delay help-seeking
- Integration into primary care under Vision 2030
- Cultural and family context is central to assessment and adherence

Frame diagnosis within the patient's cultural and religious reference frame; somatic complaints ("aches all over", fatigue) are a common idiom of depressive distress.

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PART 1

Depressive Disorders

Epidemiology and neurobiology · DSM-5-TR criteria · the full depressive spectrum · differential diagnosis · and a guideline-based approach to management.

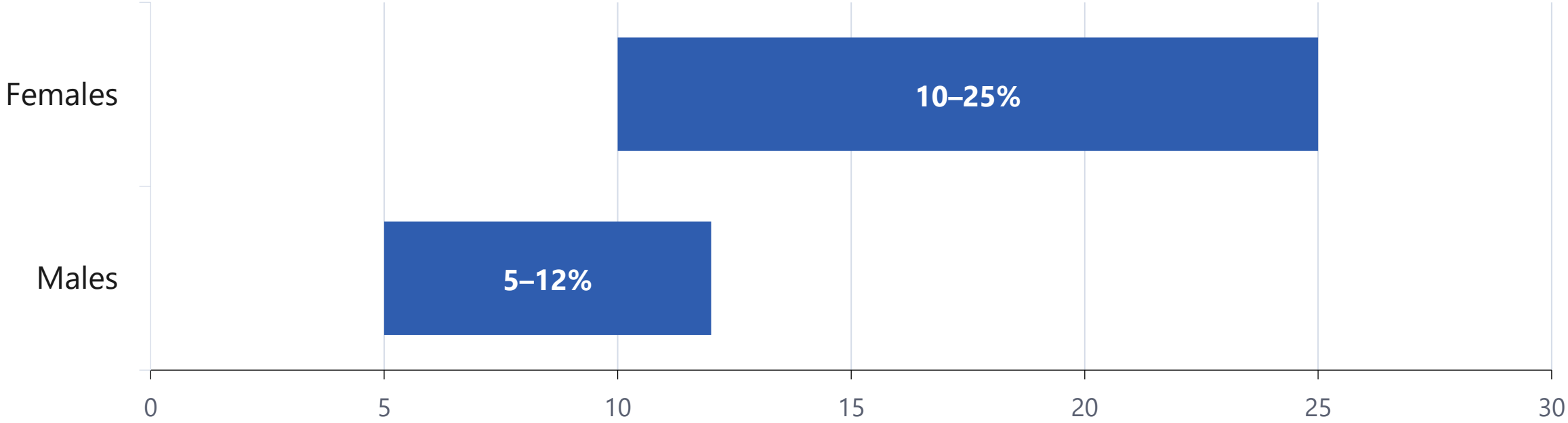
Epidemiology of depression

~280M
people affected worldwide (WHO)

2–3×
risk in first-degree relatives

1st
leading cause of disability worldwide

Reported lifetime prevalence of major depression, by sex (%)



WHO, 2023. Bars show published prevalence ranges, not point estimates. Higher-risk groups: female:male ≈ 2:1, pregnant/postpartum, divorced/widowed, low income.

Etiology — the explanatory models

No single cause explains depression. These are complementary lenses, not competing theories.

Neurotransmitter

Monoamine dysregulation — serotonin, noradrenaline, dopamine.

Genetic transmission

Heritable; increased risk in first-degree relatives; twin concordance.

Stress–diathesis

A latent vulnerability unmasked by life stress.

Neuroendocrine

HPA-axis overactivity, raised cortisol; thyroid axis.

Neuroanatomic / neurotrophic

Limbic–prefrontal dysfunction; reduced BDNF and hippocampal volume.

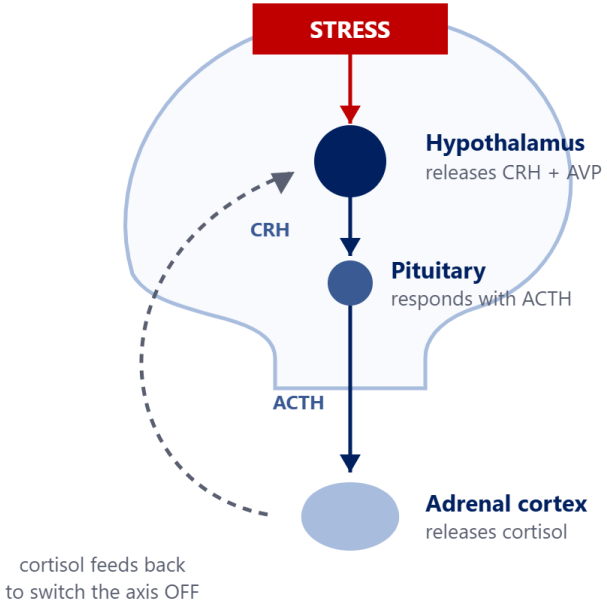
Sleep & psychological

Shortened REM latency; the negative cognitive triad; learned helplessness.

Icons: Lucide (ISC licence), rasterised on-device to BMC navy.

The neurobiology of depression

Chronic stress drives the HPA axis — and excess cortisol is toxic to the hippocampus.



What goes wrong in depression

Chronic stress keeps CRH and cortisol persistently high

Feedback fails — many patients do not suppress on dexamethasone

Sustained cortisol is toxic to the hippocampus — volume falls

BDNF and hippocampal neurogenesis drop with it

Mood lifts at 4–6 weeks, not hours — recovery needs neuroplastic adaptation

Why the delay? Monoamines rise in hours, but mood lifts in 4–6 weeks — recovery depends on slower receptor and neuroplastic adaptation. Heritability ~40% (MZ > DZ).

Why treat depression?

Systemic harm

- An independent risk factor for heart attack and stroke
- Worsens diabetes and chronic-disease outcomes
- Drives ED visits, hospitalisation and health-care use

Mortality

- 40–60% of completed suicides involve a depressed patient
- A leading global cause of disability and lost productivity

Depression is not "just low mood" — it is a systemic, potentially lethal illness with cardiovascular and suicide mortality. Effective treatment reduces both.

The differential diagnosis of sadness

Normal / reactive

- Grief and bereavement
- Difficulty coping
- Demoralisation
- Adjustment disorder

Primary mood disorders

- Major depressive disorder
- Persistent depressive disorder
- Bipolar I and II disorder
- Schizoaffective disorder

Secondary / organic

- Mood disorder due to a medical illness
- Delirium
- Cerebrovascular accident
- Substance/medication-induced

Always place a "sad" patient somewhere on this map before treating — the treatment for grief, adjustment disorder and MDD is not the same.

Major Depressive Episode — "SIG-E-CAPS"

SIG E CAPS

≥ 2 weeks of depressed mood and/or anhedonia, most of the day nearly every day, plus ≥ 4 more (≥ 5 total), causing distress or impairment; not due to a substance or medical condition.

S — Sleep

Insomnia or hypersomnia

I — Interest

Anhedonia — loss of interest or pleasure

G — Guilt

Guilt or worthlessness

E — Energy

Fatigue or loss of energy

C — Concentration

Poor concentration; indecisiveness

A — Appetite

Appetite or weight change

P — Psychomotor

Agitation or retardation

S — Suicidality

Thoughts of death, ideation, plan or attempt

DSM-5-TR, 2022. Sub-threshold pictures (fewer than five symptoms, or under two weeks) are not a major depressive episode — think adjustment or persistent depressive disorder.

Major Depressive Disorder (MDD)

Defining features

- Also called unipolar depression
- Single or recurrent major depressive episodes
- Marked distress or functional impairment
- Not due to a substance or medical condition

The bipolar gate

- No history of mania or hypomania — any such history makes it bipolar
- Psychosis or anxiety may occur — captured by specifiers
- Ask directly about past elevated mood, reduced sleep need, overactivity

Missing a hypomanic episode misclassifies bipolar as unipolar and risks antidepressant-induced switching.

Depression in children & adolescents

Presentation

- Irritability may replace sadness
- Somatic complaints; declining school performance
- Social withdrawal, boredom, school refusal

DMDD (DSM-5)

- Chronic, severe, persistent irritability
- Frequent temper outbursts with angry mood between them
- Onset before age 10; diagnosed ages 6–18

DMDD was created to reduce over-diagnosis of paediatric bipolar disorder: chronic irritability is not episodic mania.

Depression in the elderly

Key principles

- Depression is not a normal part of ageing
- Patients often minimise sadness; present with agitation
- Cognitive slowing may mimic dementia ("pseudodementia")

Risk

- Higher rate of completed suicide
- More lethal methods
- Often under-recognised behind somatic complaints

Older adults have the highest completed-suicide rates — screen actively even when mood is denied.

Screening for depression

Screening is case-finding, not diagnosis — a positive screen always goes on to a full clinical interview.

PHQ-2 first

Two questions on mood and anhedonia. If positive, go on to the full PHQ-9.

Then PHQ-9

Nine items mapping onto the DSM criteria. Grades severity and tracks response over time. Free to reproduce and translate.

Read the numbers

Adolescent screening: ~74% sensitivity, ~75% specificity. ~38% PPV but ~93% NPV — strong at ruling out, weaker at ruling in.

What it cannot do

A screen, not a diagnosis. Self-report — stigma drives under-reporting. Somatic items overlap with physical illness, pregnancy and drug effects, inflating scores in the medically ill. It does NOT screen for past mania — a high score says nothing about bipolarity.

Item 9 flags suicidal thoughts but is not a risk assessment — always assess risk directly. Cut-offs and test performance vary by population, age group and translation: use a version validated in the population you are screening.

Rating scales — severity & response

Scale	Type	Best use
PHQ-9	Self-report (9 items)	Screening, severity and monitoring — the everyday tool
HAM-D (Hamilton)	Clinician-rated	Classic research / trial severity measure
MADRS	Clinician-rated	Especially sensitive to change — antidepressant trials
BDI (Beck)	Self-report	Severity of depressive symptoms
EPDS (Edinburgh)	Self-report	Screening in the peripartum period
YMRS (Young)	Clinician-rated	Severity of mania

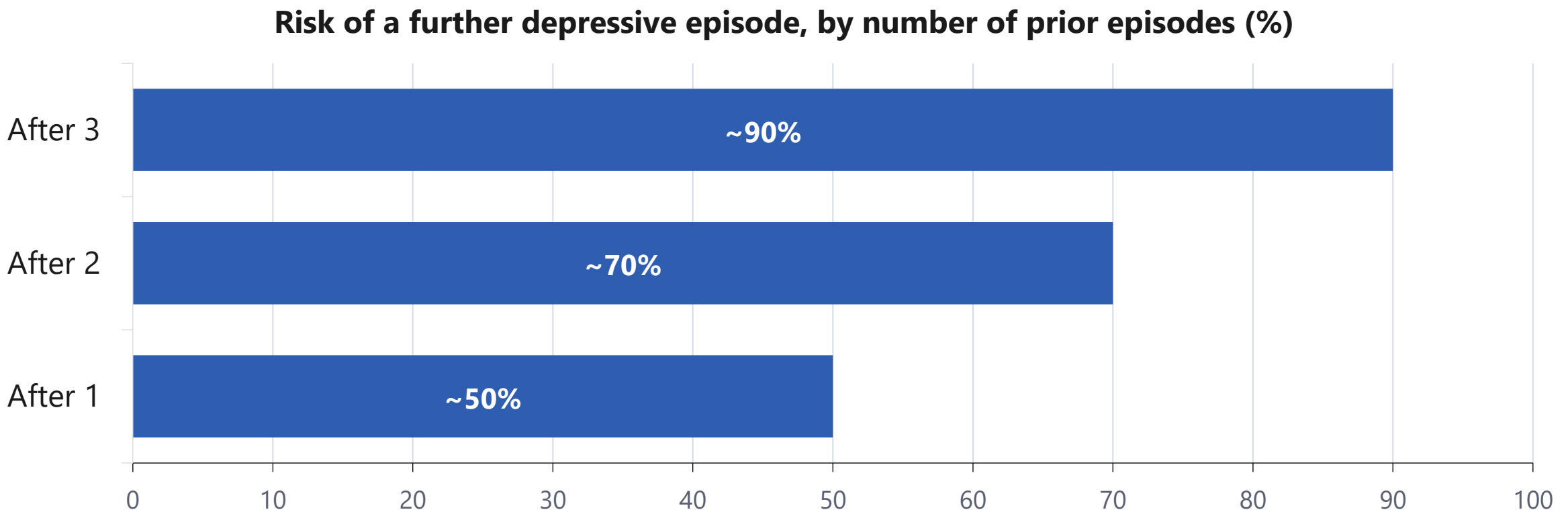
A rising or falling score across visits is objective evidence of response or relapse — but a scale never replaces the clinical interview and risk assessment.

A highly recurrent illness

6–13 mo
untreated episode; ≈ 3 months treated

~80%
recover within one year

5–10%
eventually develop bipolar disorder



Continuation: depression ≥ 6 months after the last symptoms; bipolar is typically lifelong.

Depression as a systemic illness

Cardiometabolic

- An independent risk factor for cardiovascular disease and mortality
- Increases the risk of type 2 diabetes

Associated conditions (raised rates in MDD)

- COPD/emphysema, asthma, migraine
- Stroke, epilepsy, MS, thyroid disease
- Cancer, chronic back pain

Bidirectional: chronic medical illness worsens depression and depression worsens medical outcomes — treat both.

Predictors of outcome

Lower recovery rates

- Long duration of the current episode
- Greater symptom severity
- Psychotic features
- Prominent anxiety
- Comorbid personality disorder

Higher recurrence risk

- Multiple prior episodes
- Previous severe episode
- Residual symptoms during remission
- Younger age of onset

Residual symptoms during remission are a red flag to keep treating.

Who is most at risk?

Four patients with major depression are in your clinic today.

You have time to prioritise just one for an urgent safety review.

Which factor best predicts a future suicide attempt?

A A recent stressful life event

B A previous suicide attempt

C Living alone

D Female sex

Commit to an answer before the next slide.

Who is most at risk?

ANSWER

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B **A previous suicide attempt**

C Living alone

D Female sex

B — a previous suicide attempt.

Prior suicidality is the single strongest predictor of future suicide, outweighing demographic factors. Always ask directly about past attempts and current intent, plan and access to means.

Depression & suicidality

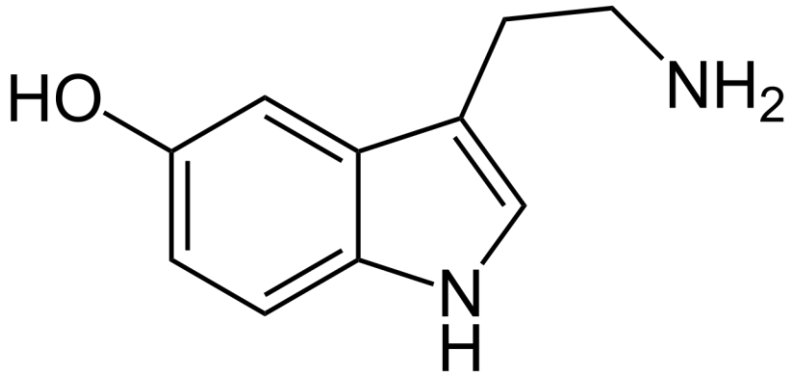
Assess ideation, intent, plan, means and protective factors at every relevant contact.

Key facts

- About two-thirds of depressed patients contemplate suicide
- Depression is involved in 40–60% of completed suicides
- Prior suicidality is the single strongest predictor

Neurobiology

- Low CSF 5-HIAA — a serotonin metabolite — is associated with suicidal behaviour
- Low CSF 5-HIAA also predicts future suicidal behaviour



Serotonin (5-HT). Jü, Wikimedia Commons, public domain

Restrict access to means and document a safety plan. Asking about suicide does not plant the idea.

How well does treatment work?

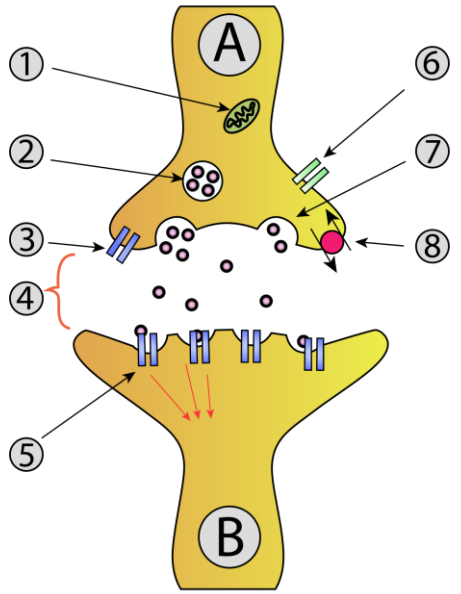
Antidepressants roughly double the placebo response — 50–65% vs 25–30% over ~3 months.

Where the drugs act

- Every antidepressant class targets the synapse in the figure
- SSRIs block the reuptake pump — item 8
- Monoamines rise within hours

Why response is delayed

- Onset of effect takes 4–6 weeks
- The effect depends on receptor and neuroplastic adaptation, not the rise itself
- ~80% recover within a year with treatment
- Continue 6–9 months after remission; longer if recurrent or high-risk



Chemical synapse — 8 is the reuptake pump, the SSRI target. Mouagip/Dake, Wikimedia Commons, CC BY-SA 3.0

Set patient expectations early: the drug is working before the patient feels it. Drug classes, dosing and monitoring are covered in the Psychopharmacology lecture.

Before you diagnose depression…

A 48-year-old woman has 3 months of low mood, fatigue, weight gain, poor concentration and constipation.

She also reports cold intolerance, dry skin and a hoarse voice.

Single most important next step?

A Start an SSRI

B Check thyroid function (TSH)

C Refer for CBT

D Reassure, review in 6 weeks

Commit to an answer before the next slide.

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D Reassure, review in 6 weeks

B — check thyroid function.

Classic hypothyroidism, which mimics depression. Always exclude a medical — and medication — cause before diagnosing primary MDD; treating the thyroid can resolve the mood without an antidepressant.

Secondary to a medical / medication cause

Medical conditions

- Hypothyroidism, Cushing's, Addison's
- Pancreatic and other cancers
- Post-stroke, post-MI
- SLE, multiple sclerosis, HIV/AIDS

Medications

- Corticosteroids, anabolic steroids
- Interferon
- Isotretinoin
- Some antihypertensives and hormonal agents

Screen thyroid function and review the medication list in every new presentation of depression.

Is this really primary depression?

A 52-year-old man is referred for "depression" — low mood, poor sleep, anhedonia and irritability for a month.

He drinks heavily most evenings, and his symptoms are worst in the mornings. No prior psychiatric history.

Most likely explanation?

A Major depressive disorder

B Substance-induced mood disorder

C Adjustment disorder

D Bipolar depression

Commit to an answer before the next slide.

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B — substance-induced mood disorder.

Heavy alcohol use — and stimulant withdrawal, opioids and others — commonly produces depressive states that lift with abstinence. Don't lock in a primary MDD diagnosis during active use; reassess after a substance-free interval, aided by a urine drug screen.

Substance-induced mood disorder

Common culprits

- Alcohol and sedative-hypnotic use or withdrawal
- Stimulant "crash" — cocaine, amphetamine
- Cannabis, opioids

Diagnostic approach

- Temporal link to use or withdrawal
- Re-assess after a period of abstinence
- Urine drug screen supports the picture

Mood symptoms driven by substances often remit with abstinence — avoid committing to a primary mood-disorder diagnosis during active use or acute withdrawal.

Premenstrual Dysphoric Disorder (PMDD)

≥ 5 symptoms in the final week before menses, improving within a few days of onset, minimal post-menses — confirmed prospectively over ≥ 2 cycles.

≥ 1 affective symptom

- Affective lability — mood swings, tearfulness, rejection sensitivity
- Irritability or anger
- Depressed mood or hopelessness
- Anxiety, tension

Plus additional symptoms

- Reduced interest, poor concentration
- Fatigue, appetite change
- Hyper- or insomnia; feeling overwhelmed
- Physical: breast tenderness, bloating

PMDD is a formal depressive disorder, not "bad PMS" — the key is timing, confined to the late luteal phase and confirmed prospectively.

Persistent Depressive Disorder (dysthymia)

Depressed mood most of the day, more days than not, for ≥ 2 years (≥ 1 year in children and adolescents).

Plus ≥ 2 of six

- Appetite change (up or down)
- Sleep disturbance — insomnia or hypersomnia
- Low energy or fatigue

...continued

- Low self-esteem
- Poor concentration or indecisiveness
- Hopelessness

Never symptom-free for more than 2 months during the 2-year period. A superimposed major depressive episode is called "double depression".

Depression specifiers — symptom-based

Specifiers are not decoration — each one changes management or prognosis.

Anxious distress

Keyed-up, tense, restless; worry-impaired focus; fear of losing control. Higher suicide risk.

Atypical

Mood reactivity (required), hypersomnia, increased appetite and weight, leaden paralysis, rejection sensitivity.

Melancholic

Non-reactive mood, anhedonia, early-morning waking, diurnal variation, marked guilt, psychomotor change.

Catatonic

≥ 2 of immobility, catalepsy, waxy flexibility, stupor, mutism, negativism — or excessive purposeless activity.

Atypical depression historically responds to MAOIs; melancholic depression responds well to medication and ECT.

Depression specifiers — course & features

With psychotic features

- 10–15% of episodes; mood-congruent or -incongruent
- Worse prognosis; raised suicide and homicide risk
- Antidepressant + antipsychotic; consider ECT early

With peripartum onset

- Onset in pregnancy or ≤ 4 weeks postpartum — 3–6% meet this DSM specifier; ~10–15% within the first year
- Distinguish transient "baby blues" (~50–80%, resolving within 2 weeks)
- Infant-focused delusions or command hallucinations = emergency

With seasonal pattern (SAD)

- Predictable seasonal onset and remission
- Not explained by seasonal psychosocial stressors
- Treat with light therapy or antidepressants



Bright-light therapy. User:Mysid, Wikimedia Commons, public domain

These course specifiers carry real management consequences — psychotic depression needs an antipsychotic and earlier ECT; peripartum psychosis is an emergency.

The postpartum spectrum

Feature	Baby blues	Postpartum depression	Postpartum psychosis
Prevalence	~50–80%	~10–15%	~1–2 per 1000
Onset	Days 3–10	Within weeks (up to 12 months)	Rapid — days to 2 weeks
Features	Mild tearfulness, lability, anxiety	Full depressive episode ± anxiety	Delusions, hallucinations, confusion, mood instability
Course	Self-limiting (≤ 2 weeks)	Persists; needs treatment	Fluctuating; deteriorates fast
Action	Reassurance, support	Psychotherapy ± antidepressant; screen (EPDS)	Emergency admission

Postpartum psychosis is a psychiatric emergency — infant-focused delusions or command hallucinations carry real risk to mother and infant. Admit urgently, never manage at home.

Grief — or depression?

Three weeks after his wife's death, a 60-year-old man has sadness that comes in waves.

He still enjoys time with his grandchildren, misses her deeply, and wishes he "could be with her" — but has no wish to end his life. His sleep is poor.

Which feature would point to MDD rather than normal grief?

A Sadness that comes in waves

B Preserved self-esteem

C Pervasive worthlessness and guilt

D Wishing to be with the deceased

Commit to an answer before the next slide.

Grief — or depression?

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B Preserved self-esteem

C **Pervasive worthlessness and guilt**

D Wishing to be with the deceased

C — pervasive worthlessness and guilt.

Normal grief preserves self-esteem, comes in waves, and allows moments of pleasure. Global worthlessness, guilt and pervasive anhedonia point to MDD — which, since DSM-5, may be diagnosed even during bereavement.

Grief & Prolonged Grief vs. MDD

Feature	Normal grief	Major depressive disorder
Affect	Emptiness and loss, in waves; may include humour and positive emotion	Persistent low mood / pervasive anhedonia
Self-esteem	Preserved	Worthlessness, global guilt
Suicidal thought	Thoughts of "joining" the deceased	Ending life from worthlessness or pain
Course & function	Eases over days–weeks; impairment transient	Sustained; needs treatment

Bereavement exclusion removed (DSM-5): MDD may be diagnosed during grief if full criteria are met. New in DSM-5-TR: Prolonged Grief Disorder (≥ 12 months).

Adjustment disorder vs. MDD

Feature	Adjustment disorder	Major depressive disorder
Trigger	Identifiable stressor within 3 months	May or may not follow a stressor
Criteria	Distress out of proportion, but does not meet full MDD criteria	Meets full ≥ 5-symptom, 2-week criteria
Duration	Resolves within 6 months of the stressor ending	Persists; recurrent
Management	Supportive; psychotherapy; problem-solving	Antidepressant ± psychotherapy

If the depressive syndrome is severe enough to satisfy full MDD criteria, diagnose MDD — not adjustment disorder — even when a clear stressor is present.

Schizoaffective disorder — DSM-5 criteria

Criteria

- A. Uninterrupted illness with a major mood episode concurrent with Criterion A of schizophrenia
- B. Delusions or hallucinations for ≥ 2 weeks without a major mood episode (lifetime)
- C. Mood symptoms present for the majority of the illness
- D. Not due to a substance or medical condition

The key discriminator

- Mood-with-psychosis (MDD/bipolar): psychosis only during mood episodes
- Schizoaffective: ≥ 2 weeks of psychosis without mood symptoms
- Schizophrenia: mood symptoms brief relative to the psychosis

The discriminator is the timing of psychosis relative to mood — a classic exam point.

Investigations & comorbidities

Routine work-up

- CBC; TSH; LFTs; electrolytes
- B12, folate
- Urinalysis + urine drug screen

Additional (per case)

- ECG — especially before TCAs
- CXR
- Neurology consult / imaging

Comorbidities

- Substance use, anxiety, eating disorders
- Borderline PD; ADHD, learning and conduct disorders
- Men → substance use; women → anxiety/eating

The work-up excludes organic mimics before committing to a primary mood disorder. Comorbidity is the rule, not the exception.

Depression — a stepped-care approach

Match the intensity of treatment to the severity of illness — the stepped-care principle.



Match treatment intensity to illness severity. Continue 6–9 months after remission; **≥ 2 years if recurrent or high-risk.**

Drug classes, dosing, side-effects and monitoring are covered in the Psychopharmacology lecture.

Psychotherapy in mood disorders

First-line for mild–moderate depression, and — combined with medication — superior to either alone in severe illness.

CBT

Restructures negative automatic thoughts and the cognitive triad with behavioural experiments. Strongest evidence base; also prevents relapse.

IPT

Time-limited; works through current relationships — grief, role transitions, role disputes, interpersonal deficits.

Behavioural activation

Re-engages the patient with rewarding activity to break the withdrawal–low-mood cycle. Effective and simple to deliver.

MBCT

Mindfulness plus cognitive skills; proven to reduce relapse in recurrent depression.

Family-Focused Therapy

Bipolar. Psychoeducation and communication training that lowers expressed emotion and relapse.

IPSRT

Bipolar. Stabilises sleep–wake and social rhythms; circadian regularity protects against mood episodes.

Neurostimulation & somatic therapies

ECT

most effective acute treatment for severe depression

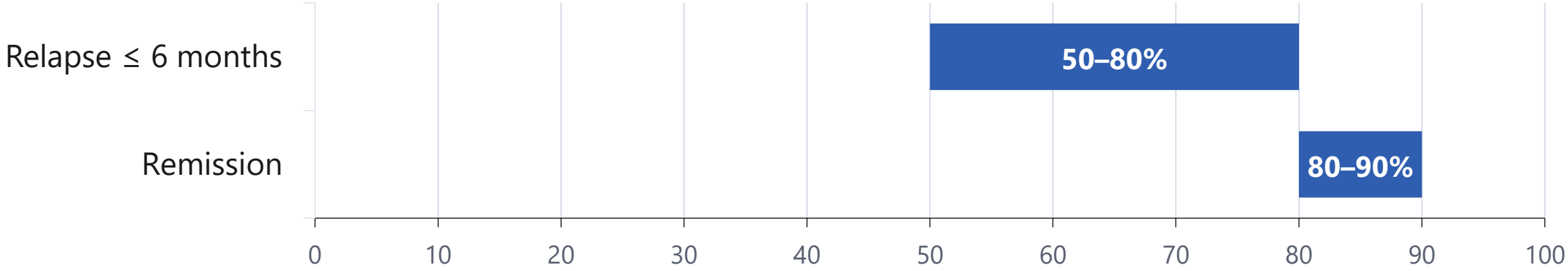
rTMS

non-invasive, outpatient, no anaesthesia — for treatment resistance

Light

10,000 lux morning exposure for the seasonal pattern

ECT in severe depression: remission, and relapse within 6 months without continuation therapy (%)



ECT is a brief, controlled generalised seizure under general anaesthesia — first-line when waiting is unsafe: intense suicidality, catatonia, psychosis, food or fluid refusal, pregnancy. Side-effects: transient memory disturbance (bilateral more effective but more cognitive load than unilateral), headache, myalgia. VNS and DBS are reserved for severe refractory illness in specialist settings.

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PAUSE

Break — 10 minutes

After the break: the other pole — mania and hypomania, bipolar I, II and cyclothymia, the differential, and management by phase — then the case, the board questions and the take-home.

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PART 2

Bipolar & Related Disorders

Mania and hypomania · bipolar I, II and cyclothymia · course, impact and differential diagnosis · and management by phase.

Mrs. R, 31 — presenting with depression

Mrs. R, 31, presents with low mood, poor appetite, poor concentration and worthlessness.

Two years ago, in the 4 days before her sister's wedding, she slept only 2 hours a night, "wasn't tired," had "brilliant ideas" racing through her mind, worked almost all night making pastries and gifts, and kept working productively at her job. Family noted she was inappropriate with men at the wedding. She denied any substance use.

Mania is the full manic pole (bipolar I). Hypomania is a milder, non-impairing high (bipolar II). Euthymia is the baseline between episodes, and depression is the depressive pole.

> **Diagnosis: Bipolar II disorder — a discrete ≥ 4-day hypomanic episode (no marked impairment, no hospitalisation, no psychosis) in a patient now presenting with major depression.**

Mania or hypomania?

A 26-year-old man has 5 days of elevated, expansive mood with a reduced need for sleep, rapid speech and increased activity.

He feels more productive than usual and is functioning well at work. There are no psychotic symptoms and no need for admission.

Which episode is this?

A Manic episode

B Hypomanic episode

C Neither — normal happiness

D Cyclothymia

Commit to an answer before the next slide.

Mania or hypomania?

ANSWER

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Which episode is this?

A Manic episode

B **Hypomanic episode**

C Neither — normal happiness

D Cyclothymia

B — a hypomanic episode.

≥ 4 days of mood change plus increased energy, but with no marked impairment, no hospitalisation and no psychosis. Add any one of those and it becomes mania. This picture points toward bipolar II.

Manic vs. hypomanic episode

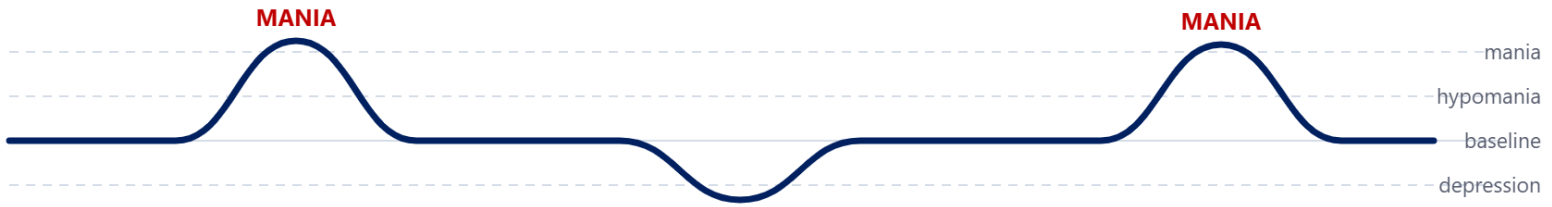
DIG FAST — Distractibility · Impulsivity/Indiscretion · Grandiosity · Flight of ideas · ↑ Activity · ↓ Sleep need · Talkativeness (pressured speech)

	Manic episode	Hypomanic episode
Duration	≥ 1 week — or any duration if hospitalised	≥ 4 consecutive days
Symptoms	≥ 3 — or ≥ 4 if mood is only irritable	Same symptom threshold
Function	Marked impairment, hospitalisation, or psychotic features	Noticeable change, but none of those three

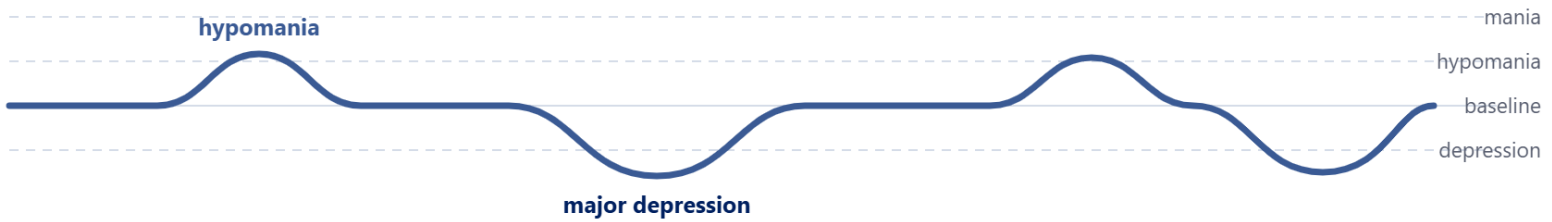
The energy gate: DSM-5 requires increased energy or activity IN ADDITION to the mood change — elevated mood alone is not sufficient.

Bipolar I vs. II vs. cyclothymia

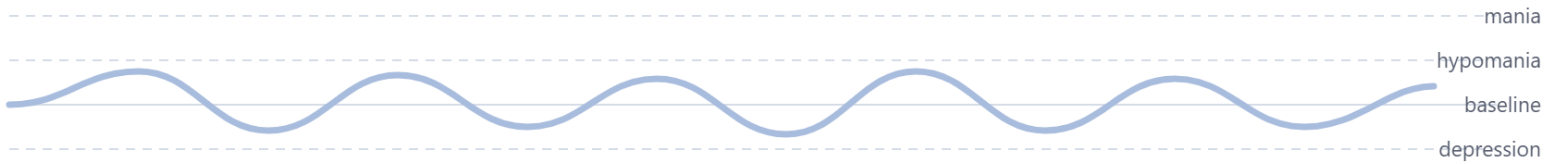
Bipolar I — ≥ 1 manic episode (depression usual, not required)



Bipolar II — hypomania + major depression, never mania



Cyclothymia — ≥ 2 years sub-threshold swings, never episode-level



Cyclothymic swings never reach an episode threshold — and are never symptom-free > 2 months.

Screen every depressed patient for past hypomania or mania before starting an antidepressant. Bipolar treatment is lifelong.

Bipolar specifiers & rapid cycling

Specifiers

- Rapid cycling · psychotic (mood-congruent or -incongruent) · melancholic · atypical · catatonic · peripartum · seasonal
- Rapid cycling: ≥ 4 mood episodes in 12 months, separated by ≥ 2 months' remission or a switch to the opposite polarity

Rapid-cycling patient profile

- Higher incidence in bipolar II
- More common in women
- Earlier onset, longer illness duration
- Higher rate of suicide attempts and morbidity

Rapid cycling is a course specifier, not a separate disorder — it flags a more severe, harder-to-treat illness.

Mixed features

A mood episode carrying ≥ 3 symptoms of the opposite pole at the same time — the two poles co-occur rather than alternate.

What it looks like

- Depressed but agitated, with racing thoughts and irritability
- Manic but tearful, hopeless, with guilt
- Easily mistaken for agitated depression or anxiety

Why it matters

- The highest suicide risk of any mood presentation
- Antidepressant monotherapy can worsen it — use a mood stabiliser or atypical antipsychotic
- Suggests bipolarity — screen carefully

Whenever depression and activation coexist, think mixed features, assess suicide risk closely, and avoid antidepressant monotherapy.

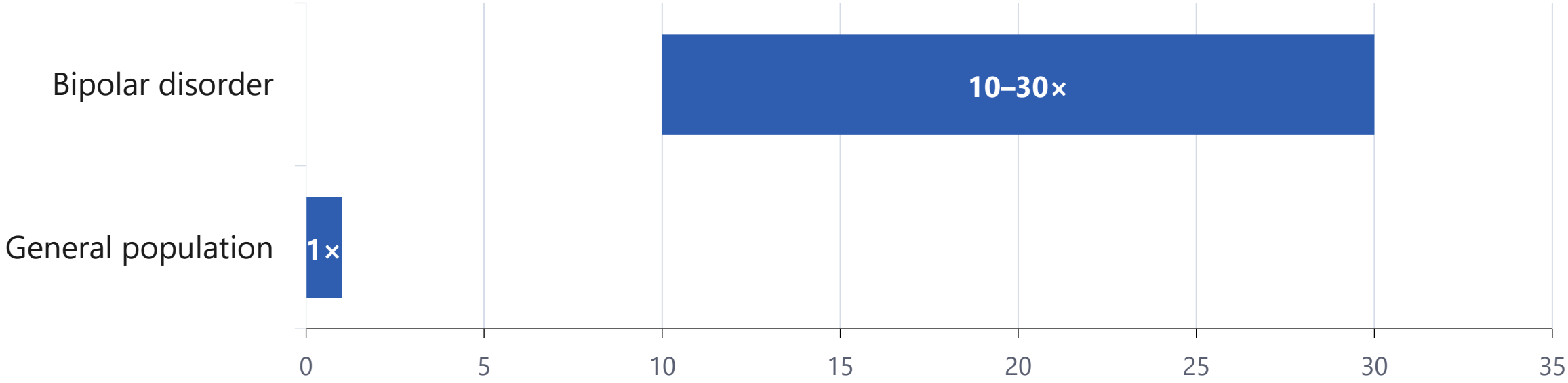
The burden of bipolar disorder

~1%
lifetime bipolar I (~2–4% bipolar spectrum)

mid-20s
typical age of onset

–12–14 yr
life expectancy (modern estimate)

Completed suicide in bipolar disorder, relative to the general population (× baseline risk)



Bipolar I is roughly equal in men and women; bipolar II is commoner in women. Strong genetic loading — the highest heritability of the mood disorders. Divorce or separation is about twice as common, with marked occupational disruption.

Prognosis & suicide risk

Predictors of poor prognosis

- Psychotic features; mixed episodes
- Rapid cycling
- Comorbid substance use
- Poor treatment adherence
- Early onset; frequent episodes

Suicidality

- Completed suicide ≈ 6–7% (older inpatient-based estimates quote 15–20%) — 10–30× the general population
- Bipolar II is particularly at risk
- Highest risk in depressive and mixed states

Mixed features — depression with activation or agitation — carry especially high suicide risk. Assess carefully and avoid antidepressant monotherapy.

What else looks like bipolar?

Consider

- Unipolar (major depressive) disorder
- Substance abuse — cocaine, amphetamine
- Personality disorders — borderline, narcissistic
- Organic / secondary mood disorders
- Schizoaffective disorder

General principle

- Establish episodicity and a clear return to baseline
- Corroborate with collateral history
- Screen for substances and medical precipitants

The unifying test is episodicity with a clear return to baseline, corroborated by collateral history.

Bipolar vs. substance, ADHD & borderline PD

Contrast	Points toward the mimic	Points toward bipolar
Substance use	Symptoms tied to intoxication or withdrawal; needs 6–12 months' abstinence to judge	Bipolar usually precedes the substance use (and is a risk factor for it)
ADHD	Onset < 12, continuous, responds to stimulants; family history of disruptive disorders	Onset later, episodic, family history of mood disorder, responds to mood stabilisers
Borderline PD	Dysfunction persists between episodes; mood often sub-threshold; history of deprivation or abuse	Biphasic episodes meeting threshold; reasonable function when euthymic; family history of bipolar

Conditions & drugs that can precipitate mania

Medical conditions

- Hyperthyroidism, Cushing's
- Multiple sclerosis, stroke
- Temporal-lobe / frontal pathology
- Neurosyphilis, HIV

Medications / substances

- Corticosteroids
- Antidepressants (switch)
- Stimulants, cocaine, amphetamine
- Dopaminergics (L-dopa), sympathomimetics

A first manic episode at an atypical age, or with focal neurological signs, warrants a work-up for a secondary or organic cause before settling on primary bipolar disorder.

Spot the dangerous shortcut

A patient with known bipolar I disorder presents with a major depressive episode.

A colleague suggests simply starting an SSRI on its own.

What is the main risk of antidepressant monotherapy here?

A It is the correct first-line treatment

B Switch into mania or rapid cycling

C Serotonin syndrome

D No particular risk

Commit to an answer before the next slide.

Spot the dangerous shortcut

A patient with known bipolar I disorder presents with a major depressive episode.

A colleague suggests simply starting an SSRI on its own.

What is the main risk of antidepressant monotherapy here?

A It is the correct first-line treatment

B **Switch into mania or rapid cycling**

C Serotonin syndrome

D No particular risk

B — a switch into mania or rapid cycling.

Bipolar depression is treated with a mood stabiliser or an atypical antipsychotic (quetiapine, lurasidone, cariprazine, or lamotrigine). Antidepressant monotherapy is avoided — if used at all, it goes on top of a mood stabiliser.

Bipolar — management by phase

Phase	Approach (decision level)	Key rule
Acute mania	Mood stabiliser (lithium or valproate) and/or an antipsychotic; short-term benzodiazepine for agitation and sleep	Stop antidepressants
Bipolar depression	Quetiapine, lurasidone, or cariprazine; lamotrigine; or lithium	Avoid antidepressant monotherapy — switch and cycling risk
Maintenance (lifelong)	Lithium first-line — gold standard and anti-suicide. Alternatives: valproate, lamotrigine, atypicals	Treat to full remission; relapse is the rule if stopped
Severe / refractory / mixed / pregnancy	ECT	Most effective for severe and mixed states

Agent selection at decision level only — doses, blood levels, side-effects and monitoring are covered in the Psychopharmacology lecture.

Keeping patients well — psychosocial care

Medication is necessary but not sufficient. Structured psychosocial care measurably reduces relapse and hospitalisation.

Psychoeducation

Teaches the illness model and early-warning signs, with a written relapse-prevention plan so patients act before a full episode.

Family-Focused Therapy

Reduces expressed emotion — household criticism and tension — and improves communication and problem-solving.

IPSRT

Regularises sleep–wake and daily routines; circadian stability is central because rhythm disruption precipitates episodes.

Lifestyle & adherence

Sleep hygiene, substance avoidance, medication adherence and monitoring — the scaffolding that prevents relapse.

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PART 3

Review & Take-Home

Board-style questions · the five golden rules · and the primary sources behind every figure in this lecture.

Ms. F, 26 — "the antidepressants stopped working"

Ms. F is a 26-year-old teacher referred with depression that keeps coming back. This is her third episode since the age of 19.

For the past two months she has had low mood, hypersomnia, increased appetite with weight gain, leaden heaviness in her limbs, and she has withdrawn from friends after what she calls "the smallest criticism".

Her GP started fluoxetine four weeks ago. Her mother says that for the last six days she has been "completely different" — sleeping about three hours a night and not tired, talking rapidly, reorganising the whole flat at 3 a.m., and she has spent most of her salary on things she did not need. She feels "finally better than well".

She has not been hospitalised, there are no hallucinations or delusions, and she is still teaching — though the head teacher has commented on her behaviour. Her maternal uncle has "manic depression" and takes lithium.

Case problem — work through it

Questions

- 1 · What specifier best describes her depressive episode?
- 2 · Is the six-day period mania or hypomania — and how do you decide?
- 3 · What is the diagnosis, and what did the earlier episodes hide?
- 4 · What went wrong with the prescribing, and what was the missed clue?
- 5 · How would you manage her now, and long term?

Model answer

- 1 · Atypical features — hypersomnia, hyperphagia with weight gain, leaden paralysis, rejection sensitivity
- 2 · Hypomania — ≥ 4 days with no marked impairment, no psychosis, no hospitalisation (any one would make it mania)
- 3 · Bipolar II. The earlier "depressive" episodes were the depressive pole of a bipolar illness
- 4 · Antidepressant monotherapy in undiagnosed bipolar disorder. The missed clue: family history plus recurrent early-onset depression
- 5 · Stop/taper the antidepressant, start a mood stabiliser; then maintenance, psychoeducation, rhythm stabilisation, suicide-risk monitoring

Hypomania is rarely volunteered because it feels good — it must be asked about directly and with a collateral informant.

Board-style questions

Questions

- 1 · 31-y-old, current depression; 2 yrs ago 4 days of ↓sleep, ↑energy, racing ideas, overspending — no hospitalisation, no psychosis. Diagnosis?
- 2 · Most dangerous antidepressant class in overdose, best avoided in an actively suicidal outpatient?
- 3 · Single best feature separating mania from hypomania?
- 4 · Mood stabiliser with the best evidence for reducing suicide?
- 5 · 22-y-old, 4 months of low mood after her father's death; symptoms fluctuate and are improving. Best fit?

Answers

- 1 · Bipolar II — discrete ≥ 4-day hypomania plus current major depression
- 2 · Tricyclic antidepressants — cardiotoxic; QRS widening, arrhythmia
- 3 · Marked impairment / hospitalisation / psychosis (≥ 1 week)
- 4 · Lithium — uniquely anti-suicidal; needs level and renal/thyroid monitoring
- 5 · Normal grief — fluctuating, improving, function preserved

DSM-5 no longer excludes MDD during grief; screen for full criteria and for Prolonged Grief Disorder if ≥ 12 months.

Summary — five things to remember

Golden rules

- MDD = ≥ 2 weeks, ≥ 5 SIG-E-CAPS symptoms, no past mania
- Always screen a depressed patient for past hypomania or mania
- Mania = mood + energy, ≥ 1 week or hospitalisation, ≥ 3 symptoms (≥ 4 if irritable)
- Lithium is gold-standard and uniquely anti-suicidal
- Assess suicide risk at every single contact



Lithium carbonate. Wikimedia Commons, public domain

If you remember nothing else from this lecture, remember these five.

References

Every statistic in this lecture was cross-checked against these primary sources; where recent evidence revised an older teaching figure, the updated value is used.

Classification & guidelines

- APA — DSM-5-TR (2022): all diagnostic criteria; Prolonged Grief Disorder
- NICE NG222 — Depression in adults (2022)
- NICE CG185 — Bipolar disorder
- APA Practice Guideline for Major Depressive Disorder

Core texts & evidence

- Boland & Verduin — Kaplan & Sadock's Synopsis of Psychiatry, 12th ed.
- Taylor, Barnes & Young — Maudsley Prescribing Guidelines, 14th ed.
- WHO — Depressive disorder fact sheet, 2023; Global Burden of Disease, 2019
- FDA — esketamine 2019/2020; zuranolone 2023 · Patorno et al., NEJM 2017

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Thank You

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Questions welcome. Sources for every figure are on the references slide.